

# INTEGRATED THERAPY & RECOVERY

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May 4, 2026

**Re: Clinical Treatment Summary - Migraine-Related Depressive Symptoms - Michael Andrew McGuire, DOB 12/24/1960**

To Whom It May Concern: I am writing at the request of Mr. Michael Andrew McGuire, who has been receiving outpatient psychotherapy at Integrated Therapy and Recovery from April 13, 2026 to the present, under VA Community Care Network referral #VA0057641040. This letter summarizes clinical observations regarding the ways his service-connected migraine headaches contribute to and exacerbate his depressive symptoms.

## **Purpose and Limitations**

This letter is based on my treatment relationship with Mr. McGuire, his self-report, my clinical observations, and the records below. It is not a forensic evaluation, a VA Compensation & Pension examination, or a disability rating assessment, and it does not constitute a formal nexus opinion. Rather, it is a supportive clinical treatment summary describing the relationship observed in treatment between chronic migraine burden and depressive symptoms.

## **Records Reviewed**

Treatment notes from sessions to date; intake screening measures; VHA Medical Documentation package transmitted with this referral; client self-report.

## **Diagnosis and Clinical Presentation**

Mr. McGuire is being treated for symptoms consistent with F43.10 Post-Traumatic Stress Disorder, unspecified, and F33.1 Major Depressive Disorder, recurrent, moderate. Reported and observed symptoms include depressed mood, reduced motivation, low energy, social withdrawal, chronic insomnia, anticipatory anxiety, and situational anxiety surrounding medical procedures. Standardized intake screening yielded scores in the moderate range for both depression and anxiety.

## **Service-Related History**

Mr. McGuire is a retired U.S. Marine Corps veteran with established service-connected migraine headaches rated at 30%. VA records also list migraine without aura and Major Depressive Disorder, recurrent, moderate. Mr. McGuire reports that recurrent migraines have contributed over time to disrupted sleep, reduced activity, social withdrawal, frustration, helplessness, and worsening low mood.

## **Clinical Opinion**

Based on the treatment relationship, clinical presentation, and records reviewed, it is my clinical opinion that Mr. McGuire's recurrent migraine headaches contribute to and exacerbate his depressive symptoms. This impression is supported by his consistent report that migraine burden interferes with sleep, activity, and social engagement; observed depressive symptoms during treatment; VA documentation of service-connected migraine headaches and Major Depressive Disorder; and the clinically recognized relationship between chronic pain/headache burden and depression. This is offered within an ongoing treatment relationship and is not a formal nexus opinion.

## **Treatment Summary**

Mr. McGuire has participated in weekly individual outpatient psychotherapy (CPT 90837) via secure telehealth since April 13, 2026. Treatment has focused on mood monitoring, behavioral activation, sleep hygiene support, coping with migraine-related limitations, and adjustment to compounding medical and family stressors. He has demonstrated strong engagement and consistently denied suicidal or homicidal ideation. Current prognosis is fair, contingent on continued treatment.

Please contact me with a valid authorization if additional information is needed.

Sincerely,

**Alexander Emile Marshi, M.A.**

Associate Marriage and Family Therapist, AMFT #159153

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